

Project Skyway — Program Requirements and Feasibility Study

Fabrikam Aerial Logistics · Internal · Confidential · Version 1.3 · August 28, 2026

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1. Executive Summary

Project Skyway expands Fabrikam's Heron H1 medical delivery service from its current Tacoma operation to a network of **60 partner hospitals** across Washington, Oregon, Colorado and Nova Scotia by the end of **September 2028**. The network moves blood products, lab specimens, vaccines and urgent pharmaceuticals between hospitals, labs and blood banks.

The total program budget is **\$38.4 million**: \$22.1 million of capital expenditure and \$16.3 million of operating expenditure over the first three years. The program is expected to reduce partner hospitals' courier costs by **35%** and to achieve a median delivery time of **22 minutes or less**, with **99.0%** cold-chain compliance.

The **pilot launches on May 4, 2027** with 8 hospitals in and around Pierce County, Washington. Ground infrastructure (SkyDock landing and charging stations) and the 24/7 network operations center are to be supplied by an external vendor; Northwind Avionics is the preferred bidder.

The feasibility team recommends proceeding, subject to three conditions: (1) FAA Part 108 BVLOS approval by March 2027, (2) qualification of a second battery supplier, and (3) steering-committee approval of any site requiring structural reinforcement.

Interviews with clinical, facilities and IT stakeholders on executive oversight highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that executive oversight the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

From a executive oversight perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. The executive oversight workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

The executive oversight workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The executive oversight workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

2. Background, Goals and KPIs

Hospitals in the target regions currently rely on ground couriers with a median transfer time of 71 minutes. Delays affect transfusion readiness and lab turnaround.

KPI	Baseline	Target	Measured by
Median delivery time	71 min (courier)	≤ 22 min	FleetSight mission log
Cold-chain compliance	94.2%	≥ 99.0%	Cargo-bay sensors
Courier cost reduction	—	35%	Hospital finance reports
Mission success rate	—	≥ 99.5%	NOC incident log
Hospitals live	1	60 by Sep 30, 2028	PMO tracker

Benchmarking against comparable programs suggests that clinical logistics outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. The clinical logistics workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that clinical logistics the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that clinical logistics outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that clinical logistics outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. From a clinical logistics perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on clinical logistics highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that clinical logistics joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on clinical logistics highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that clinical logistics consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

The clinical logistics workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. The clinical logistics workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

3. Phase Plan and Timeline

Phase	Dates	Scope	Exit criteria
0 — Discovery	Oct 1, 2026 – Jan 29, 2027	Site surveys, vendor selection, regulatory filings	Vendor contract signed; BVLOS application filed
1 — Pilot	May 4, 2027 – Aug 31, 2027	8 hospitals (SITE-01 to SITE-08), Washington	10,000 missions; KPIs met for 6 consecutive weeks
2 — Regional rollout	Sep 1, 2027 – Mar 31, 2028	32 additional hospitals (SITE-09 to SITE-40)	40 hospitals live; NOC at full staffing
3 — Full network	Apr 1, 2028 – Sep 30, 2028	20 additional hospitals (SITE-41 to SITE-60)	60 hospitals live; program hand-over to operations

Phase gates are chaired by the steering committee (Priya Ramanathan, Marcus Oyelaran, Amara Osei, and a rotating hospital representative).

Benchmarking against comparable programs suggests that program sequencing outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. The program sequencing workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that program sequencing consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. The program sequencing workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on program sequencing highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that program sequencing outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on program sequencing highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a program sequencing perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

The program sequencing workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that program sequencing joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

4. Site Assessments

Each candidate hospital was surveyed for landing options, distance to the nearest Fabrikam hub, expected daily volume and a readiness score (0-100). Sites scoring below 50 require structural or access work before a SkyDock can be installed.

4.01 Glenmoor Health Campus — Tacoma, Washington (SITE-01)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	34.0 km
Licensed beds	333
Expected deliveries/day	35
Readiness score	95
Assigned phase	Phase 1

Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. From a data governance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Survey notes — Glenmoor Health Campus (SITE-01)

From a weather perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that weather careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that weather outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. From a weather perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on weather highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. The weather workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

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4.02 Silverpine Memorial Hospital — Lakewood, Washington (SITE-02)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	30.7 km
Licensed beds	114
Expected deliveries/day	70
Readiness score	91
Assigned phase	Phase 1

The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on airspace highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

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Survey notes — Silverpine Memorial Hospital (SITE-02)

Benchmarking against comparable programs suggests that community engagement outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. The community engagement workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

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Lessons from the Q3 FY2026 operations period indicate that community engagement consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

From a community engagement perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

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4.03 Windrose Memorial Hospital — Puyallup, Washington (SITE-03)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Tacoma Hub
Distance to hub	24.9 km
Licensed beds	213
Expected deliveries/day	20
Readiness score	67
Assigned phase	Phase 1

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

The emergency procedures workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Windrose Memorial Hospital (SITE-03)

Interviews with clinical, facilities and IT stakeholders on weather highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on weather highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that weather consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that weather outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

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The weather workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that weather outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.04 Kestrel Point Memorial Hospital — Gig Harbor, Washington (SITE-04)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	21.6 km
Licensed beds	255
Expected deliveries/day	62
Readiness score	80
Assigned phase	Phase 1

From a hospital workflow perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that hospital workflow consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

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From a hospital workflow perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Kestrel Point Memorial Hospital (SITE-04)

From a hospital workflow perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that hospital workflow consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

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Benchmarking against comparable programs suggests that hospital workflow outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

The hospital workflow workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that hospital workflow consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

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4.05 Fernhaven Medical Center — University Place, Washington (SITE-05)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	38.3 km
Licensed beds	620
Expected deliveries/day	21
Readiness score	70
Assigned phase	Phase 1

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that data governance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. The data governance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

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Survey notes — Fernhaven Medical Center (SITE-05)

The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

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Lessons from the Q3 FY2026 operations period indicate that community engagement reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

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The community engagement workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that community engagement reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. From a community engagement perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

4.06 Ashbourne General Hospital — Federal Way, Washington (SITE-06)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	19.0 km
Licensed beds	153
Expected deliveries/day	76
Readiness score	85
Assigned phase	Phase 1

The staff training workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. From a staff training perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that staff training consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. The staff training workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

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Survey notes — Ashbourne General Hospital (SITE-06)

The data governance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. From a data governance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

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From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.07 Oakhollow Health Campus — Bonney Lake, Washington (SITE-07)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	9.4 km
Licensed beds	249
Expected deliveries/day	19
Readiness score	95
Assigned phase	Phase 1

Lessons from the Q3 FY2026 operations period indicate that hospital workflow the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that hospital workflow joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that hospital workflow outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that hospital workflow consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

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Survey notes — Oakhollow Health Campus (SITE-07)

Benchmarking against comparable programs suggests that maintenance outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. The maintenance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

The maintenance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. From a maintenance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

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From a maintenance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

4.08 Juniper Ridge Health Campus — Spanaway, Washington (SITE-08)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	26.3 km
Licensed beds	256
Expected deliveries/day	58
Readiness score	87
Assigned phase	Phase 1

Benchmarking against comparable programs suggests that hospital workflow outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that hospital workflow outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that hospital workflow outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

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Survey notes — Juniper Ridge Health Campus (SITE-08)

The energy use workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that energy use the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that energy use the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. The energy use workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

From a energy use perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. From a energy use perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that energy use outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that energy use careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.09 Quillmont Medical Center — Olympia, Washington (SITE-09)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	38.1 km
Licensed beds	230
Expected deliveries/day	76
Readiness score	94
Assigned phase	Phase 2

From a staff training perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. From a staff training perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

From a staff training perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that staff training outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

The staff training workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The staff training workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Survey notes — Quillmont Medical Center (SITE-09)

Benchmarking against comparable programs suggests that energy use outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. From a energy use perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

From a energy use perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that energy use consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on energy use highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that energy use consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.10 Stillwater Medical Center — Everett, Washington (SITE-10)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	28.4 km
Licensed beds	239
Expected deliveries/day	41
Readiness score	75
Assigned phase	Phase 2

The maintenance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. The maintenance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that maintenance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The maintenance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that maintenance outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that maintenance joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Survey notes — Stillwater Medical Center (SITE-10)

Lessons from the Q3 FY2026 operations period indicate that energy use joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on energy use highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that energy use reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that energy use the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that energy use outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that energy use careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. From a energy use perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on energy use highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

4.11 Halcyon General Hospital — Bellevue, Washington (SITE-11)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Tacoma Hub
Distance to hub	16.0 km
Licensed beds	338
Expected deliveries/day	55
Readiness score	78
Assigned phase	Phase 2

Interviews with clinical, facilities and IT stakeholders on weather highlighted reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. From a weather perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on weather highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. The weather workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that weather outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. From a weather perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Survey notes — Halcyon General Hospital (SITE-11)

From a hospital workflow perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. The hospital workflow workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

The hospital workflow workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that hospital workflow reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. The hospital workflow workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The hospital workflow workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a hospital workflow perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. The hospital workflow workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that hospital workflow joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.12 Amberly Community Hospital — Renton, Washington (SITE-12)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	40.0 km
Licensed beds	190
Expected deliveries/day	51
Readiness score	85
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that staff training outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. From a staff training perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on staff training highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From a staff training perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a staff training perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a staff training perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Amberly Community Hospital (SITE-12)

Benchmarking against comparable programs suggests that airspace outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on airspace highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. From an airspace perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that airspace careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on airspace highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.13 Larkspur Health Campus — Kent, Washington (SITE-13)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	37.0 km
Licensed beds	221
Expected deliveries/day	75
Readiness score	74
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that staff training outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that staff training outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that staff training outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

From a staff training perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Larkspur Health Campus (SITE-13)

Benchmarking against comparable programs suggests that maintenance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. From a maintenance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that maintenance outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The maintenance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that maintenance joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that maintenance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that maintenance the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The maintenance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that maintenance careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

4.14 Marrowgate Medical Center — Bremerton, Washington (SITE-14)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	31.3 km
Licensed beds	187
Expected deliveries/day	52
Readiness score	71
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on data governance highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Survey notes — Marrowgate Medical Center (SITE-14)

Interviews with clinical, facilities and IT stakeholders on data governance highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on data governance highlighted reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

The data governance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on data governance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that data governance the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

4.15 Bluestem Medical Center — Lacey, Washington (SITE-15)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	26.4 km
Licensed beds	527
Expected deliveries/day	14
Readiness score	92
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that energy use the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that energy use outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Survey notes — Bluestem Medical Center (SITE-15)

From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The data governance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that data governance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that data governance reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.16 Silverpine Medical Center — Auburn, Washington (SITE-16)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Tacoma Hub
Distance to hub	27.5 km
Licensed beds	564
Expected deliveries/day	76
Readiness score	91
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a airspace perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that airspace outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that airspace outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Silverpine Medical Center (SITE-16)

Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. From an airspace perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

From an airspace perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. From an airspace perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on airspace highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that airspace the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. From an airspace perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

4.17 Kestrel Point Community Hospital — Tumwater, Washington (SITE-17)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	26.3 km
Licensed beds	301
Expected deliveries/day	81
Readiness score	94
Assigned phase	Phase 2

From a data governance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that data governance reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. The data governance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Kestrel Point Community Hospital (SITE-17)

Benchmarking against comparable programs suggests that performance monitoring outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that performance monitoring outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that performance monitoring reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that performance monitoring outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that performance monitoring outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

The performance monitoring workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that performance monitoring the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that performance monitoring outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that performance monitoring joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that performance monitoring outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. From a performance monitoring perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

4.18 Glenmoor General Hospital — Shelton, Washington (SITE-18)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Tacoma Hub
Distance to hub	8.9 km
Licensed beds	507
Expected deliveries/day	24
Readiness score	65
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. From an airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. From an airspace perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. From an airspace perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Survey notes — Glenmoor General Hospital (SITE-18)

Lessons from the Q3 FY2026 operations period indicate that emergency procedures reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

The emergency procedures workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. From a emergency procedures perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

From a emergency procedures perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that emergency procedures careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

The emergency procedures workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

4.19 Amberly Medical Center — Port Orchard, Washington (SITE-19)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Tacoma Hub
Distance to hub	39.4 km
Licensed beds	285
Expected deliveries/day	19
Readiness score	84
Assigned phase	Phase 2

The staff training workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on staff training highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that staff training the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. From a staff training perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Amberly Medical Center (SITE-19)

Interviews with clinical, facilities and IT stakeholders on airspace highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that airspace outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that airspace outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From an airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that airspace careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that airspace outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

4.20 Tidewater Memorial Hospital — Centralia, Washington (SITE-20)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	7.4 km
Licensed beds	505
Expected deliveries/day	50
Readiness score	77
Assigned phase	Phase 2

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

The hospital workflow workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that hospital workflow careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

The hospital workflow workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. From a hospital workflow perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Tidewater Memorial Hospital (SITE-20)

Benchmarking against comparable programs suggests that airspace outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From an airspace perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that airspace outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. From an airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that airspace the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

4.21 Northgate General Hospital — Chehalis, Washington (SITE-21)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Tacoma Hub
Distance to hub	11.8 km
Licensed beds	589
Expected deliveries/day	77
Readiness score	74
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a staff training perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on staff training highlighted joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. The staff training workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. The staff training workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Survey notes — Northgate General Hospital (SITE-21)

Lessons from the Q3 FY2026 operations period indicate that performance monitoring consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that performance monitoring reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that performance monitoring consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that performance monitoring reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that performance monitoring outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that performance monitoring the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that performance monitoring outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.22 Halcyon Community Hospital — Aberdeen, Washington (SITE-22)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Tacoma Hub
Distance to hub	15.6 km
Licensed beds	328
Expected deliveries/day	42
Readiness score	84
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that staff training joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. From a staff training perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that staff training careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From a staff training perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

From a staff training perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on staff training highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Survey notes — Halcyon Community Hospital (SITE-22)

The emergency procedures workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

The emergency procedures workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that emergency procedures reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. From a emergency procedures perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. The emergency procedures workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

From a emergency procedures perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

From a emergency procedures perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

4.23 Copperfield General Hospital — Portland, Oregon (SITE-23)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Portland Hub
Distance to hub	21.7 km
Licensed beds	453
Expected deliveries/day	25
Readiness score	88
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on airspace highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Survey notes — Copperfield General Hospital (SITE-23)

From a airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on airspace highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

From a airspace perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on airspace highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. From a airspace perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

From a airspace perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that airspace careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

4.24 Fernhaven General Hospital — Salem, Oregon (SITE-24)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Portland Hub
Distance to hub	33.1 km
Licensed beds	100
Expected deliveries/day	19
Readiness score	70
Assigned phase	Phase 2

From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. From a community engagement perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

The community engagement workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a community engagement perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Fernhaven General Hospital (SITE-24)

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

From a data governance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that data governance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

From a data governance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

The data governance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

4.25 Oakhollow Regional Hospital — Eugene, Oregon (SITE-25)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Portland Hub
Distance to hub	13.4 km
Licensed beds	191
Expected deliveries/day	80
Readiness score	69
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that energy use reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on energy use highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that energy use outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

The energy use workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. From a energy use perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Oakhollow Regional Hospital (SITE-25)

Lessons from the Q3 FY2026 operations period indicate that energy use reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that energy use outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

The energy use workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that energy use consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that energy use outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

4.26 Halcyon Medical Center — Beaverton, Oregon (SITE-26)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Portland Hub
Distance to hub	25.6 km
Licensed beds	615
Expected deliveries/day	23
Readiness score	76
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that emergency procedures careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. The emergency procedures workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Halcyon Medical Center (SITE-26)

The data governance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on data governance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

The data governance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

4.27 Stillwater Memorial Hospital — Hillsboro, Oregon (SITE-27)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Portland Hub
Distance to hub	23.4 km
Licensed beds	402
Expected deliveries/day	12
Readiness score	66
Assigned phase	Phase 2

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a hospital workflow perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

The hospital workflow workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that hospital workflow consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a hospital workflow perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The hospital workflow workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Survey notes — Stillwater Memorial Hospital (SITE-27)

Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on data governance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on data governance highlighted careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on data governance highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that data governance the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that data governance reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on data governance highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

4.28 Juniper Ridge General Hospital — Gresham, Oregon (SITE-28)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Portland Hub
Distance to hub	29.3 km
Licensed beds	584
Expected deliveries/day	50
Readiness score	84
Assigned phase	Phase 2

From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From a emergency procedures perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Survey notes — Juniper Ridge General Hospital (SITE-28)

Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that airspace outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that airspace careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. From a airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on airspace highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

From a airspace perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on airspace highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

4.29 Oakhollow Community Hospital — Corvallis, Oregon (SITE-29)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Portland Hub
Distance to hub	30.1 km
Licensed beds	268
Expected deliveries/day	84
Readiness score	64
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Survey notes — Oakhollow Community Hospital (SITE-29)

From an airspace perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on airspace highlighted reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on airspace highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

From an airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that airspace outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.30 Quillmont General Hospital — Albany, Oregon (SITE-30)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Portland Hub
Distance to hub	29.1 km
Licensed beds	154
Expected deliveries/day	44
Readiness score	96
Assigned phase	Phase 2

The staff training workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that staff training joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on staff training highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on staff training highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that staff training joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that staff training outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Survey notes — Quillmont General Hospital (SITE-30)

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. From a data governance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.31 Marrowgate Memorial Hospital — Tigard, Oregon (SITE-31)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Portland Hub
Distance to hub	15.1 km
Licensed beds	165
Expected deliveries/day	22
Readiness score	84
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that maintenance careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that maintenance careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

From a maintenance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The maintenance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Marrowgate Memorial Hospital (SITE-31)

The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From an airspace perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that airspace outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. From an airspace perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. The airspace workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that airspace careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. From an airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.32 Tidewater Community Hospital — Lake Oswego, Oregon (SITE-32)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Portland Hub
Distance to hub	41.0 km
Licensed beds	554
Expected deliveries/day	18
Readiness score	60
Assigned phase	Phase 2

From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that energy use outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Tidewater Community Hospital (SITE-32)

Benchmarking against comparable programs suggests that staff training outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on staff training highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

From a staff training perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that staff training joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on staff training highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that staff training outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that staff training reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a staff training perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

The staff training workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that staff training outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that staff training outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that staff training outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.33 Bramblecrest Community Hospital — Newberg, Oregon (SITE-33)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Portland Hub
Distance to hub	4.0 km
Licensed beds	190
Expected deliveries/day	43
Readiness score	61
Assigned phase	Phase 2

From a staff training perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that staff training outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that staff training outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that staff training outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that staff training outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that staff training outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Survey notes — Bramblecrest Community Hospital (SITE-33)

Benchmarking against comparable programs suggests that weather outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. The weather workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that weather outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that weather outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

The weather workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that weather joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that weather careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that weather outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that weather joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that weather joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that weather outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a weather perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

4.34 Northgate Community Hospital — McMinnville, Oregon (SITE-34)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Portland Hub
Distance to hub	27.0 km
Licensed beds	427
Expected deliveries/day	82
Readiness score	90
Assigned phase	Phase 2

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

The emergency procedures workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. The emergency procedures workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Survey notes — Northgate Community Hospital (SITE-34)

Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that data governance the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

4.35 Fernhaven Health Campus — Denver, Colorado (SITE-35)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Aurora Hub
Distance to hub	22.0 km
Licensed beds	119
Expected deliveries/day	70
Readiness score	61
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that maintenance outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

The maintenance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that maintenance careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Survey notes — Fernhaven Health Campus (SITE-35)

Lessons from the Q3 FY2026 operations period indicate that weather careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that weather the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

From a weather perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a weather perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on weather highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that weather outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on weather highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on weather highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

The weather workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that weather outcomes depend on careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on weather highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The weather workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.36 Windrose Community Hospital — Aurora, Colorado (SITE-36)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Aurora Hub
Distance to hub	22.7 km
Licensed beds	614
Expected deliveries/day	43
Readiness score	74
Assigned phase	Phase 2

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that hospital workflow the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

From a hospital workflow perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that hospital workflow careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Windrose Community Hospital (SITE-36)

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. The emergency procedures workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that emergency procedures consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that emergency procedures the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. From a emergency procedures perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that emergency procedures the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

4.37 Larkspur Medical Center — Lakewood, Colorado (SITE-37)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Aurora Hub
Distance to hub	12.6 km
Licensed beds	420
Expected deliveries/day	62
Readiness score	78
Assigned phase	Phase 2

Interviews with clinical, facilities and IT stakeholders on airspace highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From an airspace perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that airspace outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Larkspur Medical Center (SITE-37)

The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that airspace the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

The airspace workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on airspace highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that airspace careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that airspace outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

From a airspace perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that airspace outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.38 Windrose Regional Hospital — Littleton, Colorado (SITE-38)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	38.7 km
Licensed beds	154
Expected deliveries/day	23
Readiness score	64
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that maintenance outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a maintenance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that maintenance outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. From a maintenance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Survey notes — Windrose Regional Hospital (SITE-38)

From a performance monitoring perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that performance monitoring careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that performance monitoring outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

The performance monitoring workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that performance monitoring outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. The performance monitoring workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that performance monitoring the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a performance monitoring perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

4.39 Fernhaven Memorial Hospital — Englewood, Colorado (SITE-39)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	40.0 km
Licensed beds	347
Expected deliveries/day	24
Readiness score	68
Assigned phase	Phase 2

Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on data governance highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. The data governance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Fernhaven Memorial Hospital (SITE-39)

Lessons from the Q3 FY2026 operations period indicate that maintenance reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that maintenance the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

The maintenance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that maintenance reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that maintenance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

From a maintenance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that maintenance outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

4.40 Ashbourne Medical Center — Arvada, Colorado (SITE-40)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Aurora Hub
Distance to hub	32.5 km
Licensed beds	160
Expected deliveries/day	60
Readiness score	92
Assigned phase	Phase 2

Lessons from the Q3 FY2026 operations period indicate that emergency procedures the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that emergency procedures joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. From a emergency procedures perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Survey notes — Ashbourne Medical Center (SITE-40)

Lessons from the Q3 FY2026 operations period indicate that maintenance the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

From a maintenance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The maintenance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that maintenance joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. The maintenance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a maintenance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that maintenance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

4.41 Oakhollow Medical Center — Westminster, Colorado (SITE-41)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Aurora Hub
Distance to hub	19.0 km
Licensed beds	526
Expected deliveries/day	40
Readiness score	86
Assigned phase	Phase 3

From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

The community engagement workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Survey notes — Oakhollow Medical Center (SITE-41)

The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From an emergency procedures perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that emergency procedures consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that emergency procedures the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

4.42 Copperfield Medical Center — Thornton, Colorado (SITE-42)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Aurora Hub
Distance to hub	24.9 km
Licensed beds	167
Expected deliveries/day	81
Readiness score	78
Assigned phase	Phase 3

Interviews with clinical, facilities and IT stakeholders on energy use highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that energy use outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a energy use perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Copperfield Medical Center (SITE-42)

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

The community engagement workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The community engagement workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

The community engagement workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a community engagement perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

The community engagement workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that community engagement joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.43 Ashbourne Memorial Hospital — Golden, Colorado (SITE-43)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	30.9 km
Licensed beds	326
Expected deliveries/day	84
Readiness score	64
Assigned phase	Phase 3

The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a data governance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that data governance the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Survey notes — Ashbourne Memorial Hospital (SITE-43)

From a energy use perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that energy use joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that energy use the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. From a energy use perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that energy use outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on energy use highlighted joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.44 Tidewater General Hospital — Boulder, Colorado (SITE-44)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Aurora Hub
Distance to hub	19.3 km
Licensed beds	159
Expected deliveries/day	55
Readiness score	64
Assigned phase	Phase 3

The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that energy use joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. From a energy use perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that energy use careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Survey notes — Tidewater General Hospital (SITE-44)

From a maintenance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that maintenance outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

The maintenance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. The maintenance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that maintenance reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a maintenance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

4.45 Bluestem Health Campus — Longmont, Colorado (SITE-45)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Aurora Hub
Distance to hub	9.1 km
Licensed beds	125
Expected deliveries/day	59
Readiness score	67
Assigned phase	Phase 3

The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that community engagement outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that community engagement outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

The community engagement workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Survey notes — Bluestem Health Campus (SITE-45)

The maintenance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that maintenance joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that maintenance careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that maintenance outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a maintenance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

4.46 Juniper Ridge Memorial Hospital — Parker, Colorado (SITE-46)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Aurora Hub
Distance to hub	31.0 km
Licensed beds	477
Expected deliveries/day	84
Readiness score	74
Assigned phase	Phase 3

The airspace workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that airspace outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that airspace joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

The airspace workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on airspace highlighted the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Juniper Ridge Memorial Hospital (SITE-46)

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that hospital workflow outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that hospital workflow outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that hospital workflow joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The hospital workflow workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. The hospital workflow workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that hospital workflow the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. From a hospital workflow perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

From a hospital workflow perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that hospital workflow consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. From a hospital workflow perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

4.47 Ridgeline Children's Hospital — Boulder, Colorado (SITE-47)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Aurora Hub
Distance to hub	33.8 km
Licensed beds	235
Expected deliveries/day	33
Readiness score	41
Assigned phase	Phase 3

Finding: The only viable landing location is the east rooftop, which requires structural reinforcement estimated at **\$1.2 million** and roughly 5 months of work. This is the lowest readiness score in the program. The site is deferred to Phase 3 and the reinforcement cost is **not** included in the program budget; a separate capital request is required.

The airspace workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that airspace reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that airspace outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Ridgeline Children's Hospital (SITE-47)

Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that energy use outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

From a energy use perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. From a energy use perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that energy use outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

4.48 Copperfield Community Hospital — Brighton, Colorado (SITE-48)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	32.3 km
Licensed beds	202
Expected deliveries/day	52
Readiness score	76
Assigned phase	Phase 3

Interviews with clinical, facilities and IT stakeholders on data governance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Survey notes — Copperfield Community Hospital (SITE-48)

Benchmarking against comparable programs suggests that community engagement outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that community engagement outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

The community engagement workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. The community engagement workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

From a community engagement perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. From a community engagement perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.49 Quillmont Memorial Hospital — Commerce City, Colorado (SITE-49)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	22.6 km
Licensed beds	592
Expected deliveries/day	46
Readiness score	93
Assigned phase	Phase 3

From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that community engagement joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that community engagement joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that community engagement outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

From a community engagement perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Survey notes — Quillmont Memorial Hospital (SITE-49)

From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. From a emergency procedures perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The emergency procedures workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. From a emergency procedures perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. From a emergency procedures perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The emergency procedures workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

4.50 Verity Community Hospital — Colorado Springs, Colorado (SITE-50)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	34.1 km
Licensed beds	543
Expected deliveries/day	40
Readiness score	90
Assigned phase	Phase 3

The performance monitoring workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on performance monitoring highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a performance monitoring perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Survey notes — Verity Community Hospital (SITE-50)

The hospital workflow workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a hospital workflow perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that hospital workflow outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Lessons from the Q3 FY2026 operations period indicate that hospital workflow careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that hospital workflow outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that hospital workflow reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on hospital workflow highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that hospital workflow outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. The hospital workflow workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

4.51 Amberly General Hospital — Broomfield, Colorado (SITE-51)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Aurora Hub
Distance to hub	38.4 km
Licensed beds	321
Expected deliveries/day	58
Readiness score	62
Assigned phase	Phase 3

Interviews with clinical, facilities and IT stakeholders on energy use highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that energy use reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Benchmarking against comparable programs suggests that energy use outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that energy use reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Survey notes — Amberly General Hospital (SITE-51)

Lessons from the Q3 FY2026 operations period indicate that maintenance careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that maintenance joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that maintenance outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on maintenance highlighted joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. From a maintenance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The maintenance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a maintenance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.52 Kestrel Point Regional Hospital — Louisville, Colorado (SITE-52)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Aurora Hub
Distance to hub	13.9 km
Licensed beds	271
Expected deliveries/day	65
Readiness score	68
Assigned phase	Phase 3

Benchmarking against comparable programs suggests that weather outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that weather outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that weather the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that weather the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

From a weather perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. From a weather perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3.

Survey notes — Kestrel Point Regional Hospital (SITE-52)

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a emergency procedures perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that emergency procedures outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that emergency procedures reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

From a emergency procedures perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that emergency procedures joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.53 Oakhollow General Hospital — Halifax, Nova Scotia (SITE-53)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Halifax Hub
Distance to hub	26.9 km
Licensed beds	256
Expected deliveries/day	50
Readiness score	91
Assigned phase	Phase 3

The weather workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. From a weather perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

From a weather perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. From a weather perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on weather highlighted consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on weather highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Survey notes — Oakhollow General Hospital (SITE-53)

Benchmarking against comparable programs suggests that data governance outcomes depend on reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on data governance highlighted the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

The data governance workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

The data governance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Benchmarking against comparable programs suggests that data governance outcomes depend on consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

4.54 Juniper Ridge Medical Center — Dartmouth, Nova Scotia (SITE-54)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Halifax Hub
Distance to hub	11.0 km
Licensed beds	523
Expected deliveries/day	58
Readiness score	95
Assigned phase	Phase 3

Lessons from the Q3 FY2026 operations period indicate that energy use careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that energy use careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that energy use reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

From a energy use perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. The energy use workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Survey notes — Juniper Ridge Medical Center (SITE-54)

From a energy use perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. From a energy use perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on energy use highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on energy use highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

The energy use workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that energy use joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6.

4.55 Bluestem Memorial Hospital — Bedford, Nova Scotia (SITE-55)

Attribute	Value
Proposed landing	ground-level pad
Nearest hub	Halifax Hub
Distance to hub	26.8 km
Licensed beds	345
Expected deliveries/day	64
Readiness score	74
Assigned phase	Phase 3

The airspace workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that airspace outcomes depend on reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that airspace consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on airspace highlighted the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. The airspace workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Survey notes — Bluestem Memorial Hospital (SITE-55)

The maintenance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3. From a maintenance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a maintenance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that maintenance consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a maintenance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that maintenance outcomes depend on joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

From a maintenance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. From a maintenance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.56 Ashbourne Regional Hospital — Sackville, Nova Scotia (SITE-56)

Attribute	Value
Proposed landing	rooftop pad
Nearest hub	Halifax Hub
Distance to hub	28.1 km
Licensed beds	329
Expected deliveries/day	18
Readiness score	66
Assigned phase	Phase 3

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. From a community engagement perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate. The community engagement workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that community engagement outcomes depend on consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Survey notes — Ashbourne Regional Hospital (SITE-56)

Interviews with clinical, facilities and IT stakeholders on energy use highlighted careful sequencing so that sites with the highest readiness scores go first. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3.

Benchmarking against comparable programs suggests that energy use outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live.

Benchmarking against comparable programs suggests that energy use outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that energy use the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. The energy use workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a energy use perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. From a energy use perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Interviews with clinical, facilities and IT stakeholders on energy use highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. The energy use workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

4.57 Larkspur Community Hospital — Truro, Nova Scotia (SITE-57)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Halifax Hub
Distance to hub	31.8 km
Licensed beds	402
Expected deliveries/day	31
Readiness score	75
Assigned phase	Phase 3

Interviews with clinical, facilities and IT stakeholders on maintenance highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. The maintenance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

From a maintenance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. The maintenance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

Lessons from the Q3 FY2026 operations period indicate that maintenance reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that maintenance outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Survey notes — Larkspur Community Hospital (SITE-57)

The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a data governance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. The data governance workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a data governance perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

From a data governance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. From a data governance perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7.

Interviews with clinical, facilities and IT stakeholders on data governance highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on data governance highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.58 Quillmont Regional Hospital — Kentville, Nova Scotia (SITE-58)

Attribute	Value
Proposed landing	parking-structure deck
Nearest hub	Halifax Hub
Distance to hub	28.3 km
Licensed beds	519
Expected deliveries/day	37
Readiness score	63
Assigned phase	Phase 3

Interviews with clinical, facilities and IT stakeholders on data governance highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that data governance joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

The data governance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. The data governance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Survey notes — Quillmont Regional Hospital (SITE-58)

Lessons from the Q3 FY2026 operations period indicate that community engagement joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that community engagement consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that community engagement outcomes depend on consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. The community engagement workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. The team recommends revisiting this assessment at each phase gate.

The community engagement workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

Lessons from the Q3 FY2026 operations period indicate that community engagement joint planning between Fabrikam operations and hospital logistics coordinators. No additional budget is required beyond what is described in Chapter 7. Interviews with clinical, facilities and IT stakeholders on community engagement highlighted the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate.

The community engagement workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that community engagement outcomes depend on consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

4.59 Halcyon Health Campus — Windsor, Nova Scotia (SITE-59)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Halifax Hub
Distance to hub	10.8 km
Licensed beds	556
Expected deliveries/day	63
Readiness score	71
Assigned phase	Phase 3

Benchmarking against comparable programs suggests that data governance outcomes depend on careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. The data governance workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that data governance reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that data governance consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that data governance careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. From a data governance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. These findings informed the phase plan in Chapter 3.

Survey notes — Halcyon Health Campus (SITE-59)

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3. From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

From a community engagement perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. The community engagement workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. The community engagement workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on community engagement highlighted the maturity of on-site facilities teams and their availability outside business hours. No additional budget is required beyond what is described in Chapter 7. Benchmarking against comparable programs suggests that community engagement outcomes depend on consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7.

The community engagement workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Lessons from the Q3 FY2026 operations period indicate that community engagement careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

From a community engagement perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that community engagement the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

4.60 Marrowgate Regional Hospital — Lunenburg, Nova Scotia (SITE-60)

Attribute	Value
Proposed landing	existing helipad
Nearest hub	Halifax Hub
Distance to hub	3.8 km
Licensed beds	309
Expected deliveries/day	76
Readiness score	84
Assigned phase	Phase 3

Benchmarking against comparable programs suggests that emergency procedures outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The emergency procedures workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3. The emergency procedures workstream reviewed current practice across partner hospitals and identified careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

From a emergency procedures perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. Detailed metrics are tracked in FleetSight dashboards after go-live. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted reliable telemetry and a single source of truth for mission status. These findings informed the phase plan in Chapter 3.

Survey notes — Marrowgate Regional Hospital (SITE-60)

From a emergency procedures perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that emergency procedures careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live.

The emergency procedures workstream reviewed current practice across partner hospitals and identified joint planning between Fabrikam operations and hospital logistics coordinators. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that emergency procedures consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that emergency procedures careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on emergency procedures highlighted reliable telemetry and a single source of truth for mission status. The team recommends revisiting this assessment at each phase gate.

The emergency procedures workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. These findings informed the phase plan in Chapter 3. Lessons from the Q3 FY2026 operations period indicate that emergency procedures the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

From a emergency procedures perspective, the feasibility team concluded that joint planning between Fabrikam operations and hospital logistics coordinators. This is consistent with the risk register in Chapter 6. The emergency procedures workstream reviewed current practice across partner hospitals and identified reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

5. Requirements Catalog

Requirements use MoSCoW priority. 'Must' requirements are mandatory for the pilot exit gate.

ID	Area	Requirement	Priority
REQ-001	Flight operations	Flight operations shall record a chain-of-custody entry at hand-off and receipt.	Must
REQ-002	Flight operations	Flight operations shall log every state transition with a UTC timestamp and the operator identity.	Could
REQ-003	Flight operations	Flight operations shall allow hospital staff to confirm receipt with a badge tap.	Must
REQ-004	Flight operations	The system shall record a chain-of-custody entry at hand-off and receipt.	Must
REQ-005	Flight operations	Flight operations shall expose health status to FleetSight every 15 seconds.	Could
REQ-006	Flight operations	Flight operations shall maintain temperature between 2 and 8 °C for the full mission.	Could
REQ-007	Flight operations	The system shall allow hospital staff to confirm receipt with a badge tap.	Must
REQ-008	Flight operations	The system shall record a chain-of-custody entry at hand-off and receipt.	Should

ID	Area	Requirement	Priority
REQ-009	Flight operations	The system shall support at least 18 take-offs and landings per hour per site.	Could
REQ-010	Flight operations	Flight operations shall record a chain-of-custody entry at hand-off and receipt.	Must
REQ-011	Flight operations	Flight operations shall reject payloads heavier than the certified maximum of 4.0 kg.	Could
REQ-012	Flight operations	The system shall maintain temperature between 2 and 8 °C for the full mission.	Could
REQ-013	Flight operations	The system shall expose health status to FleetSight every 15 seconds.	Should
REQ-014	Flight operations	Flight operations shall raise an alert to the NOC within 30 seconds of a threshold breach.	Must
REQ-015	Flight operations	Flight operations shall encrypt all data in transit using TLS 1.2 or later.	Could
REQ-016	Flight operations	The system shall reject payloads heavier than the certified maximum of 4.0 kg.	Should

ID	Area	Requirement	Priority
REQ-017	Flight operations	The system shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-018	Flight operations	Flight operations shall log every state transition with a UTC timestamp and the operator identity.	Should
REQ-019	Flight operations	Flight operations shall raise an alert to the NOC within 30 seconds of a threshold breach.	Should
REQ-020	Flight operations	The system shall support at least 18 take-offs and landings per hour per site.	Must
REQ-021	Cold chain	The cargo bay shall log every state transition with a UTC timestamp and the operator identity.	Must
REQ-022	Cold chain	The cargo bay shall encrypt all data in transit using TLS 1.2 or later.	Must
REQ-023	Cold chain	The cargo bay shall allow hospital staff to confirm receipt with a badge tap.	Could
REQ-024	Cold chain	The cargo bay shall support at least 18 take-offs and landings per hour per site.	Must

ID	Area	Requirement	Priority
REQ-025	Cold chain	The cargo bay shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-026	Cold chain	Cold-chain monitoring shall maintain temperature between 2 and 8 °C for the full mission.	Should
REQ-027	Cold chain	Cold-chain monitoring shall record a chain-of-custody entry at hand-off and receipt.	Could
REQ-028	Cold chain	Cold-chain monitoring shall expose health status to FleetSight every 15 seconds.	Should
REQ-029	Cold chain	Cold-chain monitoring shall expose health status to FleetSight every 15 seconds.	Must
REQ-030	Cold chain	Cold-chain monitoring shall raise an alert to the NOC within 30 seconds of a threshold breach.	Should
REQ-031	Cold chain	The cargo bay shall record a chain-of-custody entry at hand-off and receipt.	Could
REQ-032	Cold chain	The cargo bay shall raise an alert to the NOC within 30 seconds of a threshold breach.	Should

ID	Area	Requirement	Priority
REQ-033	Cold chain	Cold-chain monitoring shall be accessible to hospital staff without climbing ladders.	Must
REQ-034	Cold chain	Cold-chain monitoring shall encrypt all data in transit using TLS 1.2 or later.	Must
REQ-035	Cold chain	Cold-chain monitoring shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-036	Cold chain	The cargo bay shall log every state transition with a UTC timestamp and the operator identity.	Must
REQ-037	Cold chain	Cold-chain monitoring shall maintain temperature between 2 and 8 °C for the full mission.	Must
REQ-038	Cold chain	Cold-chain monitoring shall continue to operate on backup power for a minimum of 4 hours.	Could
REQ-039	Cold chain	Cold-chain monitoring shall continue to operate on backup power for a minimum of 4 hours.	Could
REQ-040	Cold chain	The cargo bay shall support remote firmware updates signed by the Restricted key store.	Must

ID	Area	Requirement	Priority
REQ-041	Hospital integration	The integration layer shall allow hospital staff to confirm receipt with a badge tap.	Must
REQ-042	Hospital integration	The integration layer shall use HL7 FHIR R4 and support Epic, Oracle Health, and MEDITECH.	Must
REQ-043	Hospital integration	The integration layer shall allow hospital staff to confirm receipt with a badge tap.	Should
REQ-044	Hospital integration	The integration layer shall record a chain-of-custody entry at hand-off and receipt.	Should
REQ-045	Hospital integration	The integration layer shall support remote firmware updates signed by the Restricted key store.	Could
REQ-046	Hospital integration	The hospital portal shall encrypt all data in transit using TLS 1.2 or later.	Should
REQ-047	Hospital integration	The hospital portal shall raise an alert to the NOC within 30 seconds of a threshold breach.	Must
REQ-048	Hospital integration	The integration layer shall raise an alert to the NOC within 30 seconds of a threshold breach.	Should

ID	Area	Requirement	Priority
REQ-049	Hospital integration	The hospital portal shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-050	Hospital integration	The integration layer shall record a chain-of-custody entry at hand-off and receipt.	Must
REQ-051	Hospital integration	The hospital portal shall record a chain-of-custody entry at hand-off and receipt.	Should
REQ-052	Hospital integration	The integration layer shall record a chain-of-custody entry at hand-off and receipt.	Must
REQ-053	Hospital integration	The hospital portal shall allow hospital staff to confirm receipt with a badge tap.	Should
REQ-054	Hospital integration	The hospital portal shall maintain temperature between 2 and 8 °C for the full mission.	Should
REQ-055	Hospital integration	The integration layer shall maintain temperature between 2 and 8 °C for the full mission.	Must
REQ-056	Hospital integration	The hospital portal shall allow hospital staff to confirm receipt with a badge tap.	Must

ID	Area	Requirement	Priority
REQ-057	Hospital integration	The integration layer shall log every state transition with a UTC timestamp and the operator identity.	Could
REQ-058	Hospital integration	The integration layer shall allow hospital staff to confirm receipt with a badge tap.	Could
REQ-059	Hospital integration	The hospital portal shall support at least 18 take-offs and landings per hour per site.	Must
REQ-060	Hospital integration	The integration layer shall maintain temperature between 2 and 8 °C for the full mission.	Should
REQ-061	Security and privacy	Access control shall be accessible to hospital staff without climbing ladders.	Could
REQ-062	Security and privacy	Access control shall be accessible to hospital staff without climbing ladders.	Must
REQ-063	Security and privacy	The platform shall log every state transition with a UTC timestamp and the operator identity.	Must
REQ-064	Security and privacy	Access control shall expose health status to FleetSight every 15 seconds.	Should

ID	Area	Requirement	Priority
REQ-065	Security and privacy	The platform shall record a chain-of-custody entry at hand-off and receipt.	Could
REQ-066	Security and privacy	Access control shall reject payloads heavier than the certified maximum of 4.0 kg.	Should
REQ-067	Security and privacy	Access control shall expose health status to FleetSight every 15 seconds.	Should
REQ-068	Security and privacy	Access control shall continue to operate on backup power for a minimum of 4 hours.	Must
REQ-069	Security and privacy	Access control shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-070	Security and privacy	The platform shall raise an alert to the NOC within 30 seconds of a threshold breach.	Could
REQ-071	Security and privacy	Access control shall maintain temperature between 2 and 8 °C for the full mission.	Should
REQ-072	Security and privacy	Access control shall log every state transition with a UTC timestamp and the operator identity.	Could

ID	Area	Requirement	Priority
REQ-073	Security and privacy	The platform shall maintain temperature between 2 and 8 °C for the full mission.	Should
REQ-074	Security and privacy	Access control shall raise an alert to the NOC within 30 seconds of a threshold breach.	Should
REQ-075	Security and privacy	Access control shall expose health status to FleetSight every 15 seconds.	Could
REQ-076	Security and privacy	Access control shall maintain temperature between 2 and 8 °C for the full mission.	Could
REQ-077	Security and privacy	The platform shall support remote firmware updates signed by the Restricted key store.	Could
REQ-078	Security and privacy	Access control shall be accessible to hospital staff without climbing ladders.	Should
REQ-079	Security and privacy	The platform shall support remote firmware updates signed by the Restricted key store.	Should
REQ-080	Security and privacy	The platform shall be accessible to hospital staff without climbing ladders.	Could

ID	Area	Requirement	Priority
REQ-081	Ground infrastructure	Each SkyDock shall encrypt all data in transit using TLS 1.2 or later.	Should
REQ-082	Ground infrastructure	Site infrastructure shall continue to operate on backup power for a minimum of 4 hours.	Must
REQ-083	Ground infrastructure	Each SkyDock shall encrypt all data in transit using TLS 1.2 or later.	Could
REQ-084	Ground infrastructure	Site infrastructure shall support at least 18 take-offs and landings per hour per site.	Should
REQ-085	Ground infrastructure	Each SkyDock shall support at least 18 take-offs and landings per hour per site.	Must
REQ-086	Ground infrastructure	Site infrastructure shall support remote firmware updates signed by the Restricted key store.	Must
REQ-087	Security and privacy	Patient manifests for Skyway missions shall be retained for 7 years to satisfy hospital accreditation audits.	Must
REQ-088	Ground infrastructure	Each SkyDock shall continue to operate on backup power for a minimum of 4 hours.	Should

ID	Area	Requirement	Priority
REQ-089	Ground infrastructure	Each SkyDock shall be accessible to hospital staff without climbing ladders.	Could
REQ-090	Ground infrastructure	Each SkyDock shall record a chain-of-custody entry at hand-off and receipt.	Could
REQ-091	Ground infrastructure	Each SkyDock shall maintain temperature between 2 and 8 °C for the full mission.	Must
REQ-092	Ground infrastructure	Each SkyDock shall expose health status to FleetSight every 15 seconds.	Must
REQ-093	Ground infrastructure	Each SkyDock shall allow hospital staff to confirm receipt with a badge tap.	Must
REQ-094	Ground infrastructure	Each SkyDock shall log every state transition with a UTC timestamp and the operator identity.	Could
REQ-095	Ground infrastructure	Site infrastructure shall expose health status to FleetSight every 15 seconds.	Must
REQ-096	Ground infrastructure	Each SkyDock shall be accessible to hospital staff without climbing ladders.	Must

ID	Area	Requirement	Priority
REQ-097	Ground infrastructure	Each SkyDock shall continue to operate on backup power for a minimum of 4 hours.	Should
REQ-098	Ground infrastructure	Each SkyDock shall reject payloads heavier than the certified maximum of 4.0 kg.	Should
REQ-099	Ground infrastructure	Each SkyDock shall support remote firmware updates signed by the Restricted key store.	Must
REQ-100	Ground infrastructure	Site infrastructure shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-101	Regulatory	The program shall encrypt all data in transit using TLS 1.2 or later.	Could
REQ-102	Regulatory	The program shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-103	Regulatory	Operations shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-104	Ground infrastructure	Each SkyDock shall support at least 18 take-offs and landings per hour per site.	Must

ID	Area	Requirement	Priority
REQ-105	Regulatory	Operations shall log every state transition with a UTC timestamp and the operator identity.	Could
REQ-106	Regulatory	Operations shall expose health status to FleetSight every 15 seconds.	Should
REQ-107	Regulatory	Operations shall support remote firmware updates signed by the Restricted key store.	Could
REQ-108	Regulatory	The program shall support at least 18 take-offs and landings per hour per site.	Could
REQ-109	Regulatory	Operations shall maintain temperature between 2 and 8 °C for the full mission.	Must
REQ-110	Regulatory	Operations shall continue to operate on backup power for a minimum of 4 hours.	Must
REQ-111	Regulatory	Operations shall allow hospital staff to confirm receipt with a badge tap.	Should
REQ-112	Regulatory	Operations shall maintain temperature between 2 and 8 °C for the full mission.	Must

ID	Area	Requirement	Priority
REQ-113	Regulatory	The program shall encrypt all data in transit using TLS 1.2 or later.	Should
REQ-114	Regulatory	Operations shall expose health status to FleetSight every 15 seconds.	Must
REQ-115	Regulatory	Operations shall encrypt all data in transit using TLS 1.2 or later.	Must
REQ-116	Regulatory	The program shall reject payloads heavier than the certified maximum of 4.0 kg.	Must
REQ-117	Regulatory	The program shall allow hospital staff to confirm receipt with a badge tap.	Must
REQ-118	Regulatory	The program shall raise an alert to the NOC within 30 seconds of a threshold breach.	Should
REQ-119	Regulatory	Operations shall be accessible to hospital staff without climbing ladders.	Could
REQ-120	Regulatory	The program shall support at least 18 take-offs and landings per hour per site.	Must

6. Risk Register

ID	Risk	Likelihood	Impact	Mitigation	Owner
R-01	FAA Part 108 BVLOS operating approval is delayed beyond March 2027.	High	High	Engage FAA early; run pilot under existing Part 107 waivers with visual observers if needed.	Amara Osei
R-02	Single-source battery supplier (Lumenor Cells) cannot scale beyond 400 packs per quarter.	Medium	High	Qualify a second supplier by Q2 2027; hold 90 days of safety stock.	Jun Nakamura
R-03	Winter icing in Nova Scotia reduces flyable hours by up to 30% from December to March.	High	Medium	Heated-propeller retrofit for NS fleet; ground courier fallback contract.	Siobhan MacNeil
R-04	Hospital EHR integration (HL7 FHIR R4) takes longer than planned at non-Epic sites.	Medium	Medium	Standard FHIR adapter plus manual order-entry fallback for the first 90 days.	Tomasz Brennan
R-05	Rooftop structural reinforcement required at low-readiness sites exceeds contingency.	Low	High	Defer lowest-readiness sites to Phase 3; request capital top-up if more than 3 sites require it.	Luis Echeverria
R-06	Community noise complaints delay corridor approvals in Boulder and Lake Oswego.	Medium	Medium	Publish noise studies; route over arterial roads; limit night flights.	Dana Whitfield
R-07	Spare-parts lead time for motors exceeds 10 weeks.	Low	Low	Monitor monthly; escalate at steering committee.	Jun Nakamura
R-08	Currency fluctuation on CAD-denominated costs.	Medium	Medium	Monitor monthly; escalate at steering committee.	Luis Echeverria
R-09	Cold-chain sensor drift above 0.5 °C.	Low	Medium	Monitor monthly; escalate at steering committee.	Jun Nakamura
R-10	Currency fluctuation on CAD-denominated costs.	Medium	Low	Monitor monthly; escalate at steering committee.	Jun Nakamura
R-11	Cyber incident affecting FleetSight availability.	Low	Medium	Monitor monthly; escalate at steering committee.	Siobhan MacNeil
R-12	Pilot staffing shortfall in Colorado.	Low	Medium	Monitor monthly; escalate at steering committee.	Siobhan MacNeil
R-13	Landing-pad permit rework after municipal code change.	Medium	Low	Monitor monthly; escalate at steering committee.	Siobhan MacNeil
R-14	Change in hospital procurement leadership.	Medium	Low	Monitor monthly; escalate at steering committee.	Amara Osei
R-15	Currency fluctuation on CAD-denominated costs.	Low	Low	Monitor monthly; escalate at steering committee.	Luis Echeverria
R-16	Pilot staffing shortfall in Colorado.	Low	Low	Monitor monthly; escalate at steering committee.	Siobhan MacNeil

ID	Risk	Likelihood	Impact	Mitigation	Owner
R-17	Cold-chain sensor drift above 0.5 °C.	Medium	Medium	Monitor monthly; escalate at steering committee.	Jun Nakamura
R-18	Cold-chain sensor drift above 0.5 °C.	Low	Medium	Monitor monthly; escalate at steering committee.	Jun Nakamura
R-19	Pilot staffing shortfall in Colorado.	Low	Medium	Monitor monthly; escalate at steering committee.	Jun Nakamura
R-20	Currency fluctuation on CAD-denominated costs.	Medium	Medium	Monitor monthly; escalate at steering committee.	Siobhan MacNeil
R-21	Pilot staffing shortfall in Colorado.	Low	Low	Monitor monthly; escalate at steering committee.	Amara Osei
R-22	Landing-pad permit rework after municipal code change.	Low	Low	Monitor monthly; escalate at steering committee.	Siobhan MacNeil
R-23	Landing-pad permit rework after municipal code change.	Low	Medium	Monitor monthly; escalate at steering committee.	Luis Echeverria
R-24	Currency fluctuation on CAD-denominated costs.	Medium	Low	Monitor monthly; escalate at steering committee.	Amara Osei
R-25	Cellular coverage gaps along corridor segments.	Low	Medium	Monitor monthly; escalate at steering committee.	Luis Echeverria

The top three risks by combined score are R-01 (BVLOS approval), R-02 (battery supply) and R-05 (structural reinforcement).

From a risk management perspective, the feasibility team concluded that reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Benchmarking against comparable programs suggests that risk management outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that risk management outcomes depend on the maturity of on-site facilities teams and their availability outside business hours. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on risk management highlighted careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Interviews with clinical, facilities and IT stakeholders on risk management highlighted consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate. Interviews with clinical, facilities and IT stakeholders on risk management highlighted consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on risk management highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that risk management careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6.

Benchmarking against comparable programs suggests that risk management outcomes depend on reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on risk management highlighted reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7.

7. Budget

Line item	Amount (USD)
SkyDock hardware and installation (60 sites)	\$9,850,000
Heron H1 fleet expansion (180 aircraft)	\$7,416,000
Software, integration and FleetSight Enterprise	\$2,634,000
Regulatory, noise studies and permitting	\$1,100,000
Contingency (capital)	\$1,100,000
Operations — years 1-3 (staff, NOC, maintenance, energy)	\$16,300,000
Total	\$38,400,000

Capital expenditure: \$22.1 million. Operating expenditure (years 1-3): \$16.3 million. The SkyDock line assumes the preferred vendor's fixed price for hardware, installation and integration.

Benchmarking against comparable programs suggests that financial planning outcomes depend on reliable telemetry and a single source of truth for mission status. No additional budget is required beyond what is described in Chapter 7. Lessons from the Q3 FY2026 operations period indicate that financial planning careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

The financial planning workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live. From a financial planning perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

From a financial planning perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. This is consistent with the risk register in Chapter 6. Lessons from the Q3 FY2026 operations period indicate that financial planning reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6.

Lessons from the Q3 FY2026 operations period indicate that financial planning the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6. The financial planning workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. This is consistent with the risk register in Chapter 6.

8. Regulatory Approach

US operations require FAA Part 108 beyond-visual-line-of-sight (BVLOS) approval; the application is to be filed by January 15, 2027. Canadian operations require a Transport Canada Level 1 Complex Operations certificate, already held by the Halifax office since 2024.

The regulatory compliance workstream reviewed current practice across partner hospitals and identified the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. From a regulatory compliance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. The team recommends revisiting this assessment at each phase gate.

The regulatory compliance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. No additional budget is required beyond what is described in Chapter 7. From a regulatory compliance perspective, the feasibility team concluded that careful sequencing so that sites with the highest readiness scores go first. The team recommends revisiting this assessment at each phase gate.

Lessons from the Q3 FY2026 operations period indicate that regulatory compliance reliable telemetry and a single source of truth for mission status. Detailed metrics are tracked in FleetSight dashboards after go-live. Lessons from the Q3 FY2026 operations period indicate that regulatory compliance consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

Interviews with clinical, facilities and IT stakeholders on regulatory compliance highlighted reliable telemetry and a single source of truth for mission status. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on regulatory compliance highlighted careful sequencing so that sites with the highest readiness scores go first. These findings informed the phase plan in Chapter 3.

From a regulatory compliance perspective, the feasibility team concluded that the maturity of on-site facilities teams and their availability outside business hours. Detailed metrics are tracked in FleetSight dashboards after go-live. The regulatory compliance workstream reviewed current practice across partner hospitals and identified consistent procedures, clear ownership and early escalation paths. Detailed metrics are tracked in FleetSight dashboards after go-live.

From a regulatory compliance perspective, the feasibility team concluded that consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6. Interviews with clinical, facilities and IT stakeholders on regulatory compliance highlighted consistent procedures, clear ownership and early escalation paths. This is consistent with the risk register in Chapter 6.

Appendix A — Flight Corridor Catalog

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-01-1	Tacoma Hub	Glenmoor Health Campus	39.0	300	Yes
C-01-2	Tacoma Hub	Glenmoor Health Campus	34.1	200	Yes
C-01-3	Tacoma Hub	Glenmoor Health Campus	41.1	300	Yes
C-02-4	Tacoma Hub	Silverpine Memorial Hospital	37.2	200	Yes
C-02-9	Tacoma Hub	Silverpine Memorial Hospital	34.7	300	Yes
C-02-2	Tacoma Hub	Silverpine Memorial Hospital	28.9	300	No
C-03-8	Tacoma Hub	Windrose Memorial Hospital	30.6	200	Yes
C-03-1	Tacoma Hub	Windrose Memorial Hospital	30.6	200	No
C-03-7	Tacoma Hub	Windrose Memorial Hospital	30.2	400	No
C-04-4	Tacoma Hub	Kestrel Point Memorial Hospital	23.6	400	Yes
C-04-8	Tacoma Hub	Kestrel Point Memorial Hospital	20.4	400	Yes
C-04-7	Tacoma Hub	Kestrel Point Memorial Hospital	26.3	400	Yes
C-05-5	Tacoma Hub	Fernhaven Medical Center	35.6	300	No
C-05-5	Tacoma Hub	Fernhaven Medical Center	47.2	300	Yes
C-05-3	Tacoma Hub	Fernhaven Medical Center	38.7	300	No
C-06-5	Tacoma Hub	Ashbourne General Hospital	23.6	300	No
C-06-5	Tacoma Hub	Ashbourne General Hospital	20.7	400	Yes
C-06-3	Tacoma Hub	Ashbourne General Hospital	23.5	400	Yes

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-07-4	Tacoma Hub	Oakhollow Health Campus	11.1	400	Yes
C-07-3	Tacoma Hub	Oakhollow Health Campus	8.9	300	Yes
C-07-1	Tacoma Hub	Oakhollow Health Campus	9.7	300	No
C-08-9	Tacoma Hub	Juniper Ridge Health Campus	32.0	300	Yes
C-08-3	Tacoma Hub	Juniper Ridge Health Campus	25.5	300	No
C-08-7	Tacoma Hub	Juniper Ridge Health Campus	24.7	300	No
C-09-4	Tacoma Hub	Quillmont Medical Center	47.5	400	No
C-09-1	Tacoma Hub	Quillmont Medical Center	40.0	400	Yes
C-09-3	Tacoma Hub	Quillmont Medical Center	44.8	400	Yes
C-10-8	Tacoma Hub	Stillwater Medical Center	31.0	400	No
C-10-4	Tacoma Hub	Stillwater Medical Center	27.7	200	Yes
C-10-1	Tacoma Hub	Stillwater Medical Center	30.5	200	Yes
C-11-4	Tacoma Hub	Halcyon General Hospital	14.8	300	Yes
C-11-5	Tacoma Hub	Halcyon General Hospital	17.9	300	Yes
C-11-3	Tacoma Hub	Halcyon General Hospital	19.4	300	No
C-12-3	Tacoma Hub	Amberly Community Hospital	49.8	300	No
C-12-5	Tacoma Hub	Amberly Community Hospital	42.5	400	No
C-12-7	Tacoma Hub	Amberly Community Hospital	42.8	300	No

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-13-9	Tacoma Hub	Larkspur Health Campus	33.5	300	No
C-13-4	Tacoma Hub	Larkspur Health Campus	43.9	300	Yes
C-13-7	Tacoma Hub	Larkspur Health Campus	33.4	200	Yes
C-14-9	Tacoma Hub	Marrowgate Medical Center	37.1	300	Yes
C-14-6	Tacoma Hub	Marrowgate Medical Center	33.5	300	No
C-14-8	Tacoma Hub	Marrowgate Medical Center	36.2	400	Yes
C-15-4	Tacoma Hub	Bluestem Medical Center	33.0	300	Yes
C-15-6	Tacoma Hub	Bluestem Medical Center	32.5	200	No
C-15-1	Tacoma Hub	Bluestem Medical Center	28.4	300	Yes
C-16-2	Tacoma Hub	Silverpine Medical Center	31.2	200	Yes
C-16-1	Tacoma Hub	Silverpine Medical Center	29.7	400	Yes
C-16-3	Tacoma Hub	Silverpine Medical Center	30.0	200	No
C-17-6	Tacoma Hub	Kestrel Point Community Hospital	31.0	200	Yes
C-17-6	Tacoma Hub	Kestrel Point Community Hospital	26.2	200	Yes
C-17-3	Tacoma Hub	Kestrel Point Community Hospital	25.0	300	No
C-18-3	Tacoma Hub	Glenmoor General Hospital	9.3	200	No
C-18-9	Tacoma Hub	Glenmoor General Hospital	10.7	300	Yes
C-18-6	Tacoma Hub	Glenmoor General Hospital	10.1	300	Yes

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-19-8	Tacoma Hub	Amberly Medical Center	36.9	400	No
C-19-3	Tacoma Hub	Amberly Medical Center	35.6	300	No
C-19-4	Tacoma Hub	Amberly Medical Center	42.3	300	Yes
C-20-6	Tacoma Hub	Tidewater Memorial Hospital	8.5	400	No
C-20-6	Tacoma Hub	Tidewater Memorial Hospital	8.3	200	No
C-20-3	Tacoma Hub	Tidewater Memorial Hospital	6.7	200	Yes
C-21-2	Tacoma Hub	Northgate General Hospital	13.8	300	Yes
C-21-6	Tacoma Hub	Northgate General Hospital	12.4	400	No
C-21-2	Tacoma Hub	Northgate General Hospital	13.7	200	No
C-22-3	Tacoma Hub	Halcyon Community Hospital	16.9	300	No
C-22-6	Tacoma Hub	Halcyon Community Hospital	17.0	400	No
C-22-3	Tacoma Hub	Halcyon Community Hospital	14.8	400	Yes
C-23-9	Portland Hub	Copperfield General Hospital	24.6	400	No
C-23-3	Portland Hub	Copperfield General Hospital	21.0	200	No
C-23-8	Portland Hub	Copperfield General Hospital	23.1	400	Yes
C-24-5	Portland Hub	Fernhaven General Hospital	32.5	400	Yes
C-24-6	Portland Hub	Fernhaven General Hospital	40.9	200	No
C-24-5	Portland Hub	Fernhaven General Hospital	40.8	200	No

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-25-2	Portland Hub	Oakhollow Regional Hospital	13.7	400	Yes
C-25-2	Portland Hub	Oakhollow Regional Hospital	14.9	200	Yes
C-25-1	Portland Hub	Oakhollow Regional Hospital	16.6	400	Yes
C-26-3	Portland Hub	Halcyon Medical Center	29.3	300	Yes
C-26-9	Portland Hub	Halcyon Medical Center	24.5	200	Yes
C-26-1	Portland Hub	Halcyon Medical Center	28.3	400	Yes
C-27-5	Portland Hub	Stillwater Memorial Hospital	28.2	200	No
C-27-6	Portland Hub	Stillwater Memorial Hospital	26.1	200	No
C-27-2	Portland Hub	Stillwater Memorial Hospital	28.3	300	No
C-28-1	Portland Hub	Juniper Ridge General Hospital	35.2	300	Yes
C-28-2	Portland Hub	Juniper Ridge General Hospital	31.1	200	No
C-28-5	Portland Hub	Juniper Ridge General Hospital	32.8	300	Yes
C-29-2	Portland Hub	Oakhollow Community Hospital	34.0	200	No
C-29-6	Portland Hub	Oakhollow Community Hospital	29.6	400	Yes
C-29-6	Portland Hub	Oakhollow Community Hospital	34.3	200	Yes
C-30-5	Portland Hub	Quillmont General Hospital	33.5	400	Yes
C-30-3	Portland Hub	Quillmont General Hospital	32.0	400	No
C-30-5	Portland Hub	Quillmont General Hospital	32.1	400	Yes

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-31-1	Portland Hub	Marrowgate Memorial Hospital	16.5	400	Yes
C-31-1	Portland Hub	Marrowgate Memorial Hospital	15.0	300	Yes
C-31-8	Portland Hub	Marrowgate Memorial Hospital	18.3	200	No
C-32-2	Portland Hub	Tidewater Community Hospital	45.4	400	No
C-32-3	Portland Hub	Tidewater Community Hospital	49.7	200	No
C-32-2	Portland Hub	Tidewater Community Hospital	50.7	400	No
C-33-5	Portland Hub	Bramblecrest Community Hospital	4.3	200	No
C-33-8	Portland Hub	Bramblecrest Community Hospital	4.3	200	Yes
C-33-5	Portland Hub	Bramblecrest Community Hospital	4.6	400	Yes
C-34-6	Portland Hub	Northgate Community Hospital	27.6	400	No
C-34-3	Portland Hub	Northgate Community Hospital	27.7	300	Yes
C-34-4	Portland Hub	Northgate Community Hospital	29.6	400	Yes
C-35-1	Aurora Hub	Fernhaven Health Campus	22.3	300	Yes
C-35-3	Aurora Hub	Fernhaven Health Campus	23.1	400	No
C-35-4	Aurora Hub	Fernhaven Health Campus	26.6	400	No
C-36-3	Aurora Hub	Windrose Community Hospital	22.4	200	No
C-36-5	Aurora Hub	Windrose Community Hospital	21.5	400	No
C-36-7	Aurora Hub	Windrose Community Hospital	22.8	200	No

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-37-2	Aurora Hub	Larkspur Medical Center	13.6	300	No
C-37-2	Aurora Hub	Larkspur Medical Center	12.5	400	No
C-37-9	Aurora Hub	Larkspur Medical Center	14.3	400	No
C-38-4	Aurora Hub	Windrose Regional Hospital	45.7	400	No
C-38-2	Aurora Hub	Windrose Regional Hospital	41.8	400	No
C-38-7	Aurora Hub	Windrose Regional Hospital	38.7	300	No
C-39-2	Aurora Hub	Fernhaven Memorial Hospital	39.6	200	No
C-39-3	Aurora Hub	Fernhaven Memorial Hospital	46.7	400	No
C-39-7	Aurora Hub	Fernhaven Memorial Hospital	36.4	400	Yes
C-40-3	Aurora Hub	Ashbourne Medical Center	35.7	300	No
C-40-7	Aurora Hub	Ashbourne Medical Center	31.4	300	Yes
C-40-6	Aurora Hub	Ashbourne Medical Center	31.1	300	Yes
C-41-6	Aurora Hub	Oakhollow Medical Center	18.1	400	Yes
C-41-3	Aurora Hub	Oakhollow Medical Center	18.7	300	No
C-41-3	Aurora Hub	Oakhollow Medical Center	18.0	300	Yes
C-42-9	Aurora Hub	Copperfield Medical Center	22.6	200	Yes
C-42-3	Aurora Hub	Copperfield Medical Center	30.9	400	Yes
C-42-9	Aurora Hub	Copperfield Medical Center	24.0	400	Yes

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-43-5	Aurora Hub	Ashbourne Memorial Hospital	37.7	400	Yes
C-43-6	Aurora Hub	Ashbourne Memorial Hospital	34.6	200	No
C-43-1	Aurora Hub	Ashbourne Memorial Hospital	35.6	400	No
C-44-2	Aurora Hub	Tidewater General Hospital	22.9	300	No
C-44-1	Aurora Hub	Tidewater General Hospital	17.9	400	No
C-44-7	Aurora Hub	Tidewater General Hospital	17.8	200	Yes
C-45-8	Aurora Hub	Bluestem Health Campus	10.4	200	Yes
C-45-2	Aurora Hub	Bluestem Health Campus	9.2	200	No
C-45-2	Aurora Hub	Bluestem Health Campus	9.4	200	No
C-46-2	Aurora Hub	Juniper Ridge Memorial Hospital	34.6	300	No
C-46-8	Aurora Hub	Juniper Ridge Memorial Hospital	28.0	200	Yes
C-46-9	Aurora Hub	Juniper Ridge Memorial Hospital	32.8	300	Yes
C-47-7	Aurora Hub	Ridgeline Children's Hospital	30.6	400	No
C-47-8	Aurora Hub	Ridgeline Children's Hospital	31.2	400	Yes
C-47-5	Aurora Hub	Ridgeline Children's Hospital	41.4	200	Yes
C-48-3	Aurora Hub	Copperfield Community Hospital	37.6	400	No
C-48-2	Aurora Hub	Copperfield Community Hospital	29.7	400	No
C-48-2	Aurora Hub	Copperfield Community Hospital	37.6	400	Yes

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-49-4	Aurora Hub	Quillmont Memorial Hospital	20.4	300	Yes
C-49-6	Aurora Hub	Quillmont Memorial Hospital	24.0	200	No
C-49-8	Aurora Hub	Quillmont Memorial Hospital	25.3	300	No
C-50-6	Aurora Hub	Verity Community Hospital	37.7	200	No
C-50-3	Aurora Hub	Verity Community Hospital	32.9	200	No
C-50-8	Aurora Hub	Verity Community Hospital	36.8	300	No
C-51-8	Aurora Hub	Amberly General Hospital	35.6	200	Yes
C-51-1	Aurora Hub	Amberly General Hospital	47.8	400	No
C-51-9	Aurora Hub	Amberly General Hospital	40.1	200	No
C-52-9	Aurora Hub	Kestrel Point Regional Hospital	16.2	400	No
C-52-8	Aurora Hub	Kestrel Point Regional Hospital	14.9	200	Yes
C-52-3	Aurora Hub	Kestrel Point Regional Hospital	16.4	300	No
C-53-3	Halifax Hub	Oakhollow General Hospital	27.0	400	No
C-53-3	Halifax Hub	Oakhollow General Hospital	32.6	400	No
C-53-4	Halifax Hub	Oakhollow General Hospital	27.2	200	No
C-54-8	Halifax Hub	Juniper Ridge Medical Center	13.6	300	No
C-54-8	Halifax Hub	Juniper Ridge Medical Center	13.2	400	No
C-54-8	Halifax Hub	Juniper Ridge Medical Center	12.0	400	No

Corridor	From	To	Length (km)	Max altitude (ft AGL)	Night ops
C-55-5	Halifax Hub	Bluestem Memorial Hospital	29.3	300	No
C-55-5	Halifax Hub	Bluestem Memorial Hospital	29.5	300	Yes
C-55-8	Halifax Hub	Bluestem Memorial Hospital	26.9	200	No
C-56-2	Halifax Hub	Ashbourne Regional Hospital	26.0	300	Yes
C-56-6	Halifax Hub	Ashbourne Regional Hospital	26.1	200	Yes
C-56-2	Halifax Hub	Ashbourne Regional Hospital	33.2	200	Yes
C-57-5	Halifax Hub	Larkspur Community Hospital	29.1	400	Yes
C-57-3	Halifax Hub	Larkspur Community Hospital	33.4	400	Yes
C-57-9	Halifax Hub	Larkspur Community Hospital	37.6	400	No
C-58-6	Halifax Hub	Quillmont Regional Hospital	27.6	300	No
C-58-8	Halifax Hub	Quillmont Regional Hospital	30.2	300	Yes
C-58-8	Halifax Hub	Quillmont Regional Hospital	35.1	300	No
C-59-8	Halifax Hub	Halcyon Health Campus	13.3	400	Yes
C-59-2	Halifax Hub	Halcyon Health Campus	12.8	400	Yes
C-59-4	Halifax Hub	Halcyon Health Campus	12.0	300	Yes
C-60-1	Halifax Hub	Marrowgate Regional Hospital	3.6	400	Yes
C-60-6	Halifax Hub	Marrowgate Regional Hospital	4.2	200	No
C-60-9	Halifax Hub	Marrowgate Regional Hospital	4.1	400	No

Appendix B — Glossary

BVLOS — Beyond visual line of sight.

SkyDock — Northwind's automated landing, charging and payload-transfer station.

NOC — Network operations center.

FHIR — Fast Healthcare Interoperability Resources, an HL7 standard.

FleetSight — Fabrikam's fleet management platform.